3 Clinical Applications

The work of the clinical psychologist, varied in both content and setting, is an especially important application of psychological knowledge to real-world concerns. Who are the clinical psychologists, where do they work, and what do they do? In 1985, 5,418 of the 60,120 psychologists of the American Psychological Association were also members of its Clinical Psychology Division. Approximately one third of all graduate students enrolled in psychology programs in American and Canadian universities are clinical psychology students. Employed in clinics, hospitals, service agencies, universities, industry, schools, the military, and private practice, the clinical psychologist provides psychotherapy for a wide variety of clients (40%),* conducts psychological testing (15%), teaches (15%), plans and carries out clinical research (10%), provides a variety of consultation services (10%), or functions as a unit or facility administrator 10 (%).

As these figures indicate, the clinical psychologist engages in a variety of applied activities, but psychotherapy as a direct service—its teaching, its supervision, its investigation—is now a major focus (as such, psychotherapy will also be the major focus of the present chapter). But this was not always the case, for in its formative and early years, psychological testing was the prime concern of the clinical psychologist. In fact, when the first psychological clinic was established (at the University of Pennsylvania) in 1896 by Lightner Witmer (who also coined the term *clinical psychology*), its major function was "mental testing" of children experiencing learning or related problems in school settings. As we described in chapter 1, the goal of most psychological testing was at first the measurement of mental abilities, that is, "intelligence." Used for this purpose, testing proved to be of very considerable applied value, especially for selection and classification purposes in America's world wars. As the early years of the century unfolded, use of psychological testing, in increasingly diverse forms, became widespread.

*Approximate percentage of time allocated to activity is based on surveys conducted by Cuca (1975); Garfield and Kurtz (1976), and Goldschmid, Stein, Weissman, and Sorrells (1969).

Twenty psychological clinics existed by 1914; hundreds exist today. In such settings, as well as in the context of private practice, industry, the schools, and elsewhere, the modern clinical psychologist develops, administers, and interprets a wide variety of intelligence, personality, vocational aptitude and interest, neuropsychological, and other specialty psychological tests.

But much more often, most of his or her professional time is spent as a psychotherapist, working with a wide variety of clients and seeking to relieve their distress, to encourage relearning, and to stimulate personal growth. At almost precisely the same time that Witmer's clinic and the role of the psychologist as psychological tester were first established, the roots of modern psychotherapy first developed. Josef Breuer and Sigmund Freud published *Studies in Hysteria* in 1895, a book in which they put forth the then revolutionary idea:

> . . . that many symptoms of their patients seemed to be symbolic attempts to express and resolve chronic conflicts that had their roots in upsetting experiences of early life. This led Freud to develop a form of treatment based on minute exploration of patients' personal histories, with emotional reliving of childhood experiences in the treatment setting. From the information thus gained he formed a theory of human nature and mental illness known as psychoanalysis, which supplied a rationale for his psychotherapy. (Frank, 1961, p. 80)

We will examine the contemporary nature of psychoanalytic psychotherapy in detail later in this chapter. For now, we simply wish to note that Freud's half-century of seminal thinking influenced the character of clinical psychology greatly, and, though such influence has waned in recent years, it is nevertheless still strongly felt. Korchin (1976) accurately captured the psychological testing and historical psychoanalytic basis of modern clinical psychology.

> Clinical psychology has roots in both the psychometric and dynamic [psychoanalytic] traditions of psychology. The psychometric tradition, emphasizing measurement and individual differences mainly in intellectual processes, was of greater prominence in the early history of the field. . . . The dynamic tradition, with concern focused on motivation, adaptation, and personality change, had its greatest impact at a later date and is represented in the concern of clinicians with personality dynamics, development, and psychotherapy. However, the two trends coexisted over the short history of psychology and intertwine in the development of clinical psychology. (pp. 41–42)

The Child Guidance Movement of the 1920s and 1930s, which led to the establishment of many new facilities for the treatment of childhood emotional disorders; the Veterans Administration's widespread employment of clinical psychologists to work in therapeutic capacities with emotionally impaired veterans, which began in the 1940s; and the major financial support provided to clinical psychology training programs beginning in the

1950s by the National Institute of Mental Health each contributed in very important ways to the growth of clinical psychology in general and to the present-day role of the clinical psychologist as a skilled and effective psychotherapist.

PSYCHOTHERAPY: DEFINITIONS, DIFFERENCES, AND COMMONALITIES

What is psychotherapy? Kazdin (1978) has usefully defined psychotherapy as " . . . any intentional application of psychological techniques by a clinician to the end of effecting sought-after personality or behavioral changes" (p. 281). Beyond such broad and general definitions, the originators of most of the 130 different forms of psychotherapy said to exist (Parloff, 1976) have emphasized, in defining their own brands of psychotherapy, the one or two ingredients or procedures that, on the surface at least, make their psychotherapy different from (and purportedly better than) most or all other brands. Thus, we find highlighted in various definitions of psychotherapy such techniques as examination and interpretation of repressed material made available by free association, dreams, slips, or analysis of the transference relationship (psychoanalytic psychotherapy); counterconditioning via reciprocal inhibition of progressive relaxation and imagined approach to feared objects (systematic desensitization therapy); disputation of irrational beliefs (rational–emotive therapy); correction of parataxic distortions (interpersonal psychotherapy); assignment of real-life roles and scripts for enactment (fixed-role therapy); paradoxical intervention and reframing (strategic family therapy); imagined presentation of feared stimuli to maximize anxiety (flooding); demonstration plus guided approach to feared stimuli (participant modeling); modeling, role playing and performance feedback (psychological skills training); and even the use of dance, art, massage, screaming, or other procedures, rituals, or activities. A number of these unique features of various psychotherapies will be examined in detail later in this chapter as we describe several of the more popular approaches to psychotherapy in current use in the United States.

In contrast with this considerable emphasis upon the diverse procedures that make psychotherapies *look* different, many recent writers have sought to better understand and better utilize the psychotherapies by emphasizing what these differences mask, that is, the qualities that *all* psychotherapies essentially share. This stress on *commonalities* is evident in the writings of Frank (1961, 1978), for example. He enumerated as central, common factors (a) an intense confiding *relationship* with a culturally sanctioned therapist; (b) a *rationale* or myth that both therapist and client believe in and that explains both the causes and the nature of the client's difficulties, as well as a

method for their remediation; (c) a strengthening of the client's *expectation* that psychotherapy will be helpful; (d) the encouragement of *corrective experiences*; and (e) the facilitation of *emotional arousal*.

Much of Frank's viewpoint, interestingly, was anticipated many years earlier by Rosenzweig who, in 1936, pointed out that all psychotherapies of that era shared an emphasis upon the therapist–patient relationship, catharsis, inspiration, and a systematic ideology. In recent years, belief in the importance of shared features for better understanding and utilizing of psychotherapies has appeared in the writings of many other psychotherapists including especially Goldfried (1982), A. P. Goldstein and Stein (1976), Lambert (1982), and Strupp (1982). It is a viewpoint that has been supported and encouraged by the outcomes of a number of studies examining the effectiveness of diverse psychotherapies. If the qualities upon which therapies differ are more important than the qualities they share, then research comparing the effectiveness of different therapies should more or less consistently show some therapies to be better than others. If it is the qualities they share that are responsible for whatever effectiveness emerges, therapies should generally be approximately equivalent in their successfulness. Although some approaches appear to be somewhat more useful than others in particular instances, the general result of outcome research strongly leans toward viewing therapies as approximately equivalent in effectiveness. Garske and Lynn (1984) summarized in this regard:

> Using this statistical method to compile and evaluate therapy effects, Luborsky et al. (1975) found that many techniques and forms of psychotherapy were effective but not much different relative to each other. . . . Smith et al. (1980) completed an exhaustive meta-analysis in which they reviewed 475 psychotherapy outcome studies. . . . In general, various forms of psychotherapy produced positive outcomes. . . . the average recipient of psychotherapy is more improved than 80 percent of those who go untreated. Therapies tended to be most effective, regardless of orientation or modality, with depressives, simple phobics, and analogue clients solicited for treatment. Other comparative effects were also difficult to demonstrate. Brief versus long-term interventions, individual versus group therapies, and experienced versus novice therapists produced similar effect sizes. (pp. 502–503)

Clearly, therapies appear to be largely equivalent in their effectiveness and, equally clearly, one highly plausible explanation of this result is that their shared effectiveness depends in very large measure on the common qualities we have begun to describe. We thus concur with Garske and Lynn (1984), who conclude:

> Specific factors give a therapy character and definition, albeit innocuous; common factors give a therapy clout. Second, it is assumed that many apparently specific factors are really common factors in disguise. Therapies do similar things in seemingly unique ways. For example, the process of interpretation in psychoanalysis and the exposure to increasing levels of fear-inducing stimuli

while relaxed during systematic desensitization are both effective because they expose a client to an anxiety-evoking thought that he or she was avoiding. (p. 505)

Given this considerable theoretical and experimental evidence in support of the importance of common features for better understanding and employing psychotherapies, we may now return to our earlier question, What is psychotherapy? and answer it more fully by describing in greater detail what we view as the core commonalities constituting much of the essence of all psychotherapies.

CORE COMMONALITIES

Expectancies

Clients come to psychotherapies of all types bringing with them *pretherapy expectations* about what will happen and how well it will work. For each client, such expectations grow from many cultural and personal sources including mass media portrayals of psychotherapy (in soap operas, magazines, etc.); heresay about psychotherapy in general as well as about particular therapies and therapists from friends, relatives, and others; information from their own past experiences, such as relevant books they have read, psychology courses they have taken, or previous counseling or psychotherapy they've undergone; and specific details from the person or agency who referred them to the psychotherapist they are about to meet. Once this meeting takes place, *early psychotherapy expectations* begin to form. These first impressions stem mostly from the manner, appearance, and apparent competence of the psychotherapist. As the therapy sessions unfold—three, four, five—these early bases for forming expectations give way to *results-based expectations*. Is it working? Am I feeling better? Am I less fearful, less anxious, more assertive, more self-confident, better able to eat, less depressed, or improved on some other dimension? Positive or negative answers to such crucial questions, therefore, come to be the primary shapers of patient expectations. Thirty years of research has convincingly demonstrated that pretherapy, early therapy, and results-based patient expectancies can each be an important influence on therapeutic outcome, and it is clearly appropriate to include patient expectancies among the core commonalities being considered here.

Beginning with D. Rosenthal and Frank's article in 1956, "Psychotherapy and the Placebo Effect," applied psychologists have been actively studying the nature, role, and influence of this core commonality across several different approaches to psychotherapy. Two major classifications of patient expectations have been examined: prognostic and role. Prognostic expectancies concern outcome. Is psychotherapy going to be, or anticipated to be,

helpful, effective, successful? Role expectancies are relevant to the therapy process itself. What will I be expected to do, how will my therapist behave, what are my rights and responsibilities as a patient? For both types of expectancies, a number of important research findings have emerged. For prognostic expectancies, that is, how successful the patient expects his or her therapy to be, clearly the most important research result is that of several investigations that have shown that such prognostic expectations of change influence the degree of change that actually takes place (Friedman, 1963; A. P. Goldstein, 1959, 1960; Lipkin, 1954; Marcia, Rubin, & Effran, 1969; Tollinton, 1973). It has also been shown that patient prognostic expectancies are influenced by both the referral source (Heine & Trosman, 1960) that helped steer the patient to the particular therapist and the degree of distress the patient experiences (Frank, 1978). Prognostic expectancies, in turn, influence not only degree of change as noted previously, but also the duration of therapy (A. P. Goldstein, 1962; Overall & Aronson, 1962) and the patient's willingness to be open to and influenced by the psychotherapist (A. P. Goldstein, 1971; Wilson & Evans, 1977). Positive prognostic expectancies, in all psychotherapies, could well be an especially central ingredient in establishing and carrying forward a positive therapeutic outcome. As Lambert (1982) remarked in this regard:

> . . . expectations may play a role in determining therapy success in the following sequence: The client's faith in the therapist that is fostered by the relationship leads to an increase in the client's expectations for a positive outcome from therapy. These increased expectations, in turn, persuade the client to follow the therapist's lead in trying out new behaviors. Bandura found empirical support for his hypothesis that such behavior change increases self-efficacy expectation,—that is, the expectations of personal capability. . . . Increased self-efficacy expectations, in turn help maintain the behavior change. (p. 92)

Role expectations in psychotherapy—what you expect to do and what you believe will be expected of you—have also been shown to be an important aspect of diverse psychotherapies. Some role-expectation research has sought to identify different types of expectations and to examine how they develop and what they lead to. Thus, Apfelbaum (1958) and A. P. Goldstein and Heller (1960) have investigated the place of Nurturant, Critic, and Model role expectations. Guidance (i.e., tell me what to do) versus Participation (i.e., let's figure it out together) patient role expectations are a second useful breakdown (Schofield, 1964). Several research findings have been reported using role-expectation categories such as these, but the most useful are those several studies showing the crucial importance of congruence or mutuality of the role expectations held by the patient and those held by the psychotherapist for the successful course of the treatment (Appel, 1960; Chance, 1959; Lennard & Bernstein, 1960). As the last authors observed:

> When both members of a dyad are in general agreement regarding their reciprocal obligations and returns, there is a consensus or similarity of expectations, and harmony or stability occurs in their interpersonal relations. . . . But when there is any degree of discrepancy or lack of consensus between the participants, and their expectations are dissimilar . . . manifestations of strain appear in their interpersonal relations. If expectations are too dissimilar, the system disintegrates unless the differences can be reconciled. (p. 153)
>
> If the patient expects to make rapid progress within a few sessions and does not learn that this expectation is unrealistic, he may become so discouraged that he terminates treatment. . . . If he expects the therapist to be a parent figure toward him . . . he may terminate treatment when the disappointment reaction sets in, or he may enter a stage of resistance until his misconceptions are exposed and corrected. If a patient expects to tell the therapist only those things of which he is proud and hides those things of which he is ashamed, the patient will make little progress until he learns to do otherwise. Thus, unrealistic role expectations tend to increase strain and destroy equilibrium. (p. 118)

Finally, with regard to role expectations, note should also be made of the important work of Jerome Frank and his research group (Hoehn-Saric et al., 1964) on developing and evaluating the Role Induction Interview. This is a set of pretherapy interview and instructional procedures, used successfully with a number of different types of patients to correct erroneous role expectations and effectively socialize them into accurate sets of beliefs and anticipations about what occurs in psychotherapy and what is expected of them for positive outcomes to be more likely.

These several investigations, examining the impact on both the therapy process and the outcome of patient prognostic and role expectations, clearly demonstrate such expectations to be a significant core commonality in defining what psychotherapy is and in utilizing it most effectively.

Therapist–Patient Relationship

Essentially all descriptions of psychotherapeutic approaches, of whatever theoretical orientation, place considerable emphasis upon the potency of the therapist–patient relationship for both the enhancement of the process of therapy itself and the bringing about of patient change. As we have noted elsewhere:

> . . . perhaps the major responsible ingredient in determining whether such change occurs is the quality of the helper-client relationship. The same conclusion emerges if one examines descriptions of almost all of the many different approaches to formal psychotherapy. These several approaches vary in many respects—therapist activity and directiveness, how much focus is upon behavior versus the patient's inner world of feelings and attitudes, whether emphasis is placed upon the patient's present life or his childhood history, which aspects of his current difficulty are examined, and in a host of procedural ways. Yet, almost every approach to psychotherapy emphasizes the impor-

value of the therapist-patient relationship for patient change. The better the relationship: (1) the more open is the patient about his feelings, (2) the more likely he is to explore these feelings deeply with the helper, (3) the more likely he is to listen fully to and act upon any advice offered him by the helper. (A. P. Goldstein, 1971, p. 19)

This is a widely held view. Much of the clout in diverse psychotherapies lies in the relationship, whether the therapy is psychoanalytic (Ferreira & Rosen, 1982), client-centered (Rice, 1982), cognitive (Arnkoff, 1982), behavioral (R. J. Morris & Magrath, 1982), gestalt (Greenberg, 1982), family (Stahmann & Harper, 1982), or group (Fuhriman & Barlow, 1982). What is the patient–therapist relationship? Several key ingredients are frequently mentioned, one cluster of which could be described as attraction, liking, and warmth, experienced reciprocally patient to therapist, therapist to patient. In addition to these emotional components, a good therapist–patient relationship also captures and responds to the role of the helper as a competent expert. Thus, one often finds mentioned such key relationship ingredients as patient perception of the therapist as trustworthy, credible, and skilled in the task. It appears, therefore, that a good therapist–patient relationship can best be defined as a relationship fully characterized by attitudes and behaviors reflecting both warmth and respect (A. P. Goldstein, 1971; Lambert, 1982).

In their continuing effort to better understand how psychotherapy works, and to make it work better, applied psychologists have conducted many investigations of the therapist–patient relationship, focusing in particular on its association with patient improvement. Does a good therapist–patient relationship actually lead to a better outcome for the patient, as so many psychotherapists claim? The answer appears to be a confident yes. The scientific study of psychotherapy began in 1950, and, throughout its approximately 35-year history, there have been many research demonstrations of the relationship–outcome association (Bown, 1954; Goldstein, 1971; J. Hunt, Ewing, LaForge, & Gilbert, 1959; Lambert, 1982; Parloff, 1961; Vander Veen, 1965).

The potency of the therapeutic relationship for patient change can be demonstrated not alone by research such as that just cited showing that a good relationship leads to positive patient change. The relationship–outcome connection can be further demonstrated by showing that a poor relationship results not in patient improvement but in deterioration. Lambert (1982) disclosed that:

> . . . the recognition that some patients are actually made worse by the therapy that was intended to help them has proved even more informative about the active ingredients in psychotherapy. Empirical research has helped us to identify several specific ways in which people are made worse by or hurt in therapy. Reviews of this phenomenon . . . suggest that, although some negative out-

comes may be the result of a misapplication of specific techniques, by and large they are more clearly associated with the quality of the therapeutic relationship. (p. 17)

Concretely, patient deterioration has been found to result from such relationship deficiencies as a therapist who is rejecting, manipulative, or low in her or his own interpersonal skills (Lambert, 1982), is intrusive or aggressively confronting (Yalom & Lieberman, 1971), displays low energy levels or low interest in the patient (Ricks, 1974), or makes serious errors of technique (Sandell, 1981). Indeed, psychotherapy can be for better or for worse, and a crucial determiner of which occurs is the therapist–patient relationship.

How does a good psychotherapeutic relationship develop, and can concrete steps actually be taken to improve it in situations where its quality is threatened? Observations offered by Korchin (1976) are helpful in these regards.

> What does the therapist do? Above all, he listens with unswerving attention, sympathetic concern, and continuous effort to understand the patient's personal meanings. The capacity to listen without responding in terms of one's own needs and feelings or the demands of social convention is . . . the fundamental requisite of effective psychotherapy. At appropriate moments, the therapist communicates his understanding to the patient or otherwise acts to relieve the patient's suffering. The clinician values the patient's integrity as a person and his striving for self-betterment, nor does he fault the patient for his inadequacies. At the same time, he maintains necessary objectivity and detachment. The therapist is simultaneously compassionate and dispassionate. . . . The blend of these characteristics gives the psychotherapeutic relationship its special character as a setting within which emotional relearning can take place. (pp. 284–285)

But what of the situation in which such positive relationship attitudes and behaviors by the psychotherapist are insufficient? What can be done to optimize the therapist–patient relationship when the patient is resistive to psychotherapy and feels neither warmly nor respectfully toward the psychotherapist? Means for improving the quality of the therapeutic relationship do exist and include those examined in our own research program on the enhancement of "psychotherapeutic attraction" (A. P. Goldstein, 1971, 1980). As Figure 3.1 summarizes, several interventions derived originally from social psychological laboratory studies of interpersonal attraction were used by us with resistive or low-relatability patients. In this extended research program, we were able to show that, with certain major types of patients, most of the relationship enhancers employed did increase patient liking, respect, or trust of the therapist, with positive consequences for both the therapy process (e.g., communication, openness, persuasability) and the therapy outcome.

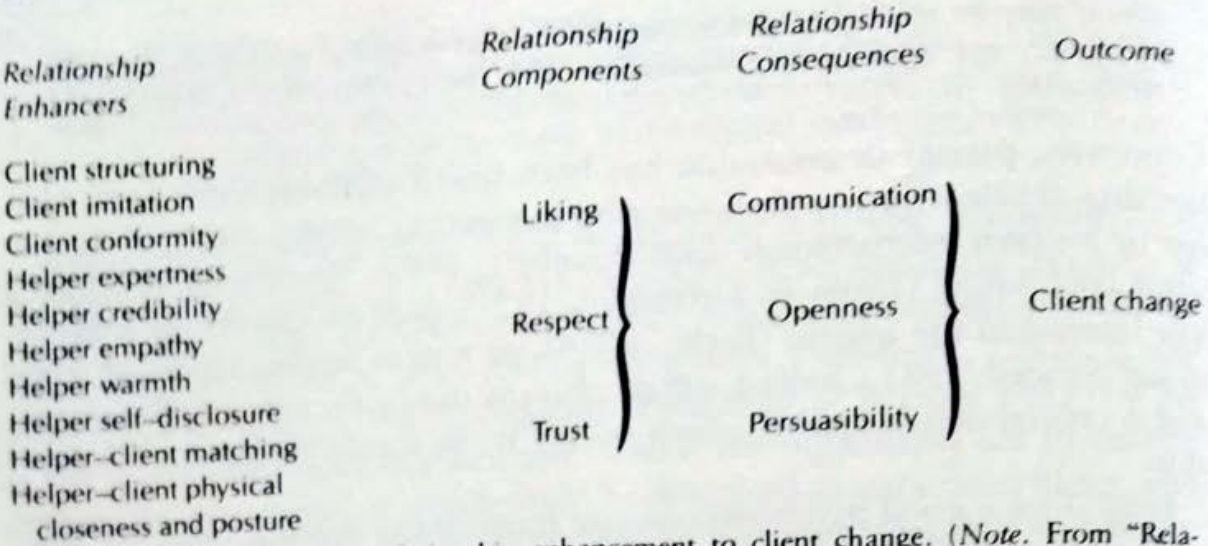


FIGURE 3.1. Progression from relationship enhancement to client change. (*Note.* From "Relationship-Enhancement Methods" by A. P. Goldstein in *Helping People Change* (3rd ed., p. 20) edited by F. H. Kanfer and A. P. Goldstein, 1986, Elmsford, NY: Pergamon. Copyright 1986 by Pergamon)

In 1959, Bordin stated:

> The key to the influence of psychotherapy on the patient is in his relationship with the therapist. Wherever psychotherapy is accepted as a significant enterprise, this statement is so widely subscribed to as to become trite. Virtually all efforts to theorize about psychotherapy are intended to describe and explain what attributes of the interactions between the therapist and the patient will account for whatever behavior change results. (p. 235)

As this section has made clear, research on both therapeutic gain and deterioration amply supports Bordin's (1959) assertion. Further, as the work of Korchin (1976) and A. P. Goldstein (1971) noted earlier suggests, the ways in which good therapeutic relationships naturally develop, or can be promoted to develop, are also largely known today. In combination, these various theoretical and research advances make it unequivocally clear that the therapist–patient relationship is indeed a most central core commonality.

New Learning: By Rationale, Modeling, and Exposure

In our view, psychotherapy is in its essence a relearning experience. The "lesson plans" can vary greatly, as can the modes of teaching and what is taught, but, nonetheless, all psychotherapies share a major focus upon the patient's learning new attitudes and behaviors. Such new learning begins with the given therapy's *rationale*, as communicated by the therapist to the patient.

> The conceptual framework of the psychotherapy facilitates this [positive] outcome by providing both an explanation of the behavioral anomaly and a prescription for change. All therapies also provide for ample success experiences in understanding those thoughts and actions that had been confusing, and in coping with stresses and anxieties that had been avoided and dealt with

> maladaptively. The patient is provided with new opportunities to learn different ways of thinking, feeling, and behaving. This is accomplished in his or her interaction with the therapist by means of interpretations, confrontations, role playing exercises, and so forth. Such novel learning opportunities are also provided outside of the therapeutic setting as homework assignments, behavioral recording, and self-monitoring. Therapeutic learning experiences, guided within a theoretical perspective by an expert practitioner solidify the patient's sense of mastery. (Garske & Lynn, 1984, p. 507)

Frank (1978) has similarly emphasized the relationship-enhancing and other positive consequences of providing the patient with a rationale, a belief system, or a "myth" about the therapy process, especially to the degree that *both* therapist and patient believe in, or come to believe in, this shared rationale.

A second shared avenue of new learning, common to all psychotherapies, is the therapist's role as a *model*. His or her attitudes, beliefs, problem-solving behaviors, language, and mannerisms may each be explicitly or implicitly modeled by the therapist and observed and imitated by the patient. Such modeling might be intentional, as in the psychological-skills-training therapies (A. P. Goldstein, 1981) or unintentional (but nonetheless as potent), as in the psychodynamic therapies (Strupp, 1982). Writing about therapist modeling as a commonality that cognitive–behavior therapy shares with other approaches, Arnkoff (1982) observed:

> In all therapies, the therapist becomes a model for the client of adaptive behavior, whether or not such modeling is planned by the therapist. . . . In the client's demoralized state, he or she may lack adequate role models, as Space and Cromwell found for depression. As the therapist persuades the client of the therapeutic rationale, the client may adopt not only the therapist's rationale and therapeutic procedures but also other aspects of the therapist's behavior, such as how the therapist responds to problems that came up in therapy. In an active therapy like cognitive therapy, the therapist displays problem-solving behavior that the client would adopt. When such problem-solving by the therapist is adaptive, cognitive therapy may be particularly strong in this factor. (p. 105)

Perry and Furukawa's (1986) extensive review of theory and research on modeling in diverse psychotherapies appropriately concluded that not only is its direct or implicit implementation widespread, but also, based on empirical evidence, it appears to play a particularly potent role in contributing to patient change.

Learning of new attitudes and behaviors by the patient can also be accomplished in many psychotherapies by a process of *exposure*. Exposure as a core commonality has been promoted most actively by Marks (1978), who explained, with regard to its potency in diverse behavioral therapies:

> The principle of exposure involves relief from phobias and compulsions by the individual's continued contact with those situations that evoke discomfort (the

> evoking stimuli, ES) until it subsides. . . . Most behavioral approaches to the treatment of anxiety syndromes like phobic and obsessive–compulsive disorders employ the common principle of exposure to the ES. What at first sight seem to be widely different forms of fear reduction—for example, desensitization in fantasy, flooding in vivo, cognitive rehearsal, modeling, and operant conditioning—all appear to be ways of exposing the patient to the frightening situation until he or she gets used to it. This process could also be called adaptation, extinction, or habituation. (p. 498)

Marks (1978) has marshalled considerable evidence in support of his assertion of exposure as a broadly potent commonality. We agree that it appears to play such a central role in several behavioral therapies and would even extend its probable importance to a less obvious but perhaps equally important significance in a number of primarily nonbehavioral therapies. Exposure is a core commonality that influences outcome, we propose, in psychodynamic therapy when interpretation, free association, and lowering of defenses permit previously warded-off contents into the patient's awareness. Exposure appears to be salient in those several cognitively oriented therapies that train and encourage patients to engage in making self-statements or self-instructions, for example, rational emotive therapy, stress inoculation, and cognitive restructuring. And exposure is perhaps most clearly evident in those instigation therapies that by script or other real-world homework assignments strongly encourage direct patient exposure to and mastery of real-life evoking stimuli. Thus it appears clear that by means of an understandable rationale for both the patient's distress and the treatment process, by the modeling of constructive behaviors by the therapist, and by exposing the patient to typically avoided sources of distress, considerable opportunity for learning new attitudes and behaviors occurs. These three common avenues for new learning do not exhaust the possibilities. The core commonality of new learning can also be implemented in many other ways such as by persuasion (Frank, 1961), suggestion (Frank, 1978), skills training (Goldstein, 1981), and problem solving (D'Zurilla & Goldfried, 1971).

Reinforcement

A final core commonality characteristic of all psychotherapies is reward, or reinforcement, provided to the patient by the psychotherapist. Some therapies, especially the contingency-management approaches, consist of procedures explicitly concerned with optimizing the effectiveness of the provision or withdrawal of reinforcement. Reinforcement scheduling, shaping, extinction, fading, response cost, and token economies are all contingency-management procedures specifying the amount, rate, scheduling, timing, or other aspects of reinforcement delivery. A great deal of evidence supports the

conclusion that contingency management can be a rapid and effective means of altering a wide variety of patient behaviors (Agras, 1967; Bandura, 1969; Kazdin, 1977; S. G. O'Leary, 1976; J. A. Sherman, 1965). Reinforcement by the therapist operates just as powerfully in psychodynamic, client-centered or other approaches, almost all of which tend to deny such a prominent influencing role to reinforcement. Even in the absence of tangible rewards for changed behavior, all therapists have been shown to provide the very powerful social reinforcement of attention to those therapy topics they wish explored further and to those patient behaviors they aspire to alter. As early as 1948, M. G. Gill and Brennan (1948) noticed that the

> . . . raw data in psychotherapeutic research is inevitably influenced by the therapist's views . . . the subtleties of showing interest in certain kinds of material, often not consciously detected either by therapist or patient, are manifold. This may include . . . a shifting of visual focus, a well-timed "mm-hmm," a scarcely perceptible nod, or even a clearing of the throat. (Krasner, 1962, p. 63)

Krasner (1955) pointed out that such subtle use of therapist reinforcement was common to all psychotherapies and, in a 1962 paper titled "The Therapist as a Social Reinforcement Machine" reviewed a wealth of evidence in support of the reinforcing potency of therapist attention and related behaviors. Evidence corroborating this conclusion has continued to accumulate in subsequent years (D. W. Johnson & Matross, 1977; Strupp, 1977). It is indeed a well-substantiated conclusion that therapist reinforcement of patient behavior is a core commonality across psychotherapies.

We have focused in this section on four core commonalities: enhanced and clarified expectations, a good therapist–patient relationship, the learning of new attitudes and behaviors, and reinforcement provided the patient by the psychotherapist. Although other possible common features across diverse therapies could eventually prove just as central—such as therapist communication skills (A. P. Goldstein & Stein, 1976; Schofield, 1964), therapist power or status (A. P. Goldstein & Stein, 1976; Schofield, 1964), the "asocial and unengaged" stance of the therapist (Beier, 1966), patient dependency and suggestibility (Frank, 1978), and the emotional arousal of the patient (Frank, 1961)—we believe the four we have emphasized constitute a very major part of the answer to What is psychotherapy?

MAJOR PSYCHOTHERAPIES

It is certainly the case that different approaches to psychotherapy look quite different from one another. Therapists of different orientations seem to be doing very different things to, with, and for their patients. It is not known at the present whether these considerable between-therapies differences in technique are merely equivalent (and in a sense interchangeable) ways of express-

ing the same core commonalities, or whether they are differences that have at least some effect by, in fact, adding therapeutic clout to that provided by the core commonalities themselves. This latter view—that the core commonalities are the main factors bringing about patient change, but that technique differences between therapies add somewhat to this effectiveness—does not seem to be an unreasonable stance to take. This is especially true in light of evidence that has begun to slowly grow in recent years that some psychotherapies could be prescriptively more effective with some types of patients, and not with others (A. P. Goldstein & Stein, 1976; Magaro, 1969). If psychotherapies were in fact completely equivalent or transitive, if *only* what they shared were responsible for patient change, there would be little or no support for the prescriptive conclusion now emerging that, at least for some types of patients, one type of therapy actually is more effective than another type. In the remainder of this chapter, by means of both factual text and constructed case histories, we will describe in detail a number of the major psychotherapies in current use today, in conjunction with the type of patient problem for which each appears most prescriptive. In doing so, we will highlight both the manner in which each therapy expresses the core commonalities described earlier and the particular technique differences that give each approach its unique character.

Cognitive–Behavior Therapy

The Golner Center for Psychotherapy

The Golner Center for Psychotherapy was established in 1954 as a clinic whose purpose was to provide psychotherapy for large numbers of patients, regardless of their ability to pay. Both its founders and its staff of 13 full-time psychotherapists were clinical psychologists. Anticipating the research evidence on the value of conducting psychotherapy prescriptively that began to appear in the 1970s, the staff of psychotherapists were diverse in their preferred approaches to psychotherapy, unlike in most other clinics, then and now, which are mostly psychoanalytic, or mostly behavioral, or mostly some other particular orientation. More and more as years passed, which patient was assigned to which therapist for which psychotherapy was an intake and diagnostic decision that the Golner staff sought to make as prescriptively as possible.

On May 2, Verna Hill, age 28, white, single, and obviously very unhappy, presented herself at Golner for psychotherapy, accompanied (really escorted) by her sister. The intake interview and psychological testing revealed all the classic signs of a serious depression. Verna had been sleeping and eating poorly and had lost almost 15 pounds in the last 3 months. She tended to move around slowly, seemingly using great effort for even minor chores. She had little interest in many of the events going on around her and, when she did communicate with others, as often as not it was to express her feelings of worthlessness and guilt for behaviors most other people would not find troublesome. Her sister reported that on two occasions Verna had spoken with

a sense of determination of trying to injure herself and, in fact, had been increasingly careless in the past month when using their kitchen stove and, before her sister had prohibited all driving, when driving their automobile.

Cognitive–behavior therapy currently appears to be the prescriptive psychotherapy of choice for adults experiencing depressive disorders. Growing out of rational–emotive therapy (Ellis, 1977), as well as the mainstream of the behavioral approaches, cognitive–behavior therapy was developed primarily by Beck (1976) and his co-workers. Much of what causes and maintains depression, according to Beck, is distorted thinking by the individual, especially in his or her underlying assumptions and information processing. The kinds of thinking that are held to characterize depression include:

1. *Arbitrary inference*, which is drawing conclusions when the evidence for the conclusion is actually contrary or absent. A frown on the face of a passerby results in the thought "He is upset with me or disgusted by me."

2. *Overgeneralization*, which is reaching broad conclusions based on very little data. In the example of the misunderstood frown just given, overgeneralization would involve adding the thought, "Just like most people are disgusted by me."

3. *Magnification and minimization*, which are the tendencies to distort by, respectively, exaggerating or catastrophizing the importance of a (possibly negative) event or downplaying or denying the importance of a (possibly positive) event. The depressed college student magnifies the negative implications of her C grade in biology, and denies or minimizes the perhaps counterbalancing implications of her A grades in chemistry and physics.

4. *Dichotomous reasoning*, which is the tendency to think in extremes or absolutes, in a polarized, all-or-none manner. The person must be perfect or totally good; the future is all black, with no hope; failure is seen as total, with no redeeming features.

It is this pattern of assumptions and thinking errors, therefore, that leads to the characteristic focus of depressed individuals on loss, a sense of inadequacy, expectation of negative outcomes and, consequently, leads to depressed emotions. As Hollon and Beck (1979), described it:

> . . . they regard themselves as deprived, defeated or diseased; their worlds as full of roadblocks to their obtaining even minimal satisfaction; and their futures as devoid of any hope of gratification and promising only pain and frustration. (p. 154)

Cognitive–behavior therapy consists of procedures that seek to alter these fundamental, erroneous assumptions and patterns of thinking. First, the individual is encouraged to monitor and record his or her negative automatic thoughts. This is a data-collection phase for the therapist (How much? Of what type?) and an increasing-awareness phase for the patient. The second phase, best labeled *rationale-expectancies relationship*, to reflect the core

commonalities it seeks to establish, is captured well by Meichenbaum (1985):

> Over the course of treatment the therapist begins to impart to the client a conceptual framework for understanding the client's maladaptive behaviors. The translation process fosters a sense of positive anticipation and hope in clients, and replaces the sense of helplessness and hopelessness. . . . A useful way to accomplish this translation process is to establish a collaborative working relationship between the client and the therapist. (p. 270)

Given this base of self-monitored information about disordered assumptions and thoughts, the provision of a rationale for change, and the beginnings of both positive prognostic expectancies and a positive therapist–patient relationship, the more didactic, instructional phases of cognitive-behavior therapy can proceed. Here, the therapist actively seeks to alter the patient's guiding assumptions, negative self-statements, and self-defeating behaviors. A variety of procedures is used to accomplish these goals. The therapist "arranges, encourages, challenges, cajoles, and persuades" (Meichenbaum, 1985, p. 270) the client to try out new positive behaviors under circumstances in which the outcomes are likely to refute negative assumptions. These are, in a sense, real-world "experiments" (exposures) the patient has typically not attempted on her or his own. By means of *reattribution* and *decentering* techniques, faulty assumptions are further challenged. Alternative, more balanced, and optimistic assumptions are proposed, and they are tested in both role play and real-life contexts. To increase the likelihood that these more balanced ways of thinking and more optimistic prognostications about the outcome of events prove accurate, the client might also be provided with new learning via interpersonal skills training to improve effectiveness and satisfaction in interactions with others. Research evidence available regarding the effectiveness of cognitive–behavior therapy for aiding depressed individuals is quite positive (Beck, 1976; R. Miller & Berman, 1981).

Psychodynamic Psychotherapy

Phil Beyer received the Junior Executive of the Year Award from his company last year, and it was clear that both his bosses and his co-workers saw him as a man on the way up. Phil was bright, very hard working, full of good ideas, and he had scores of friends. It looked more and more as if he'd someday become the first black vice president Bellweather Corporation ever had.

In the last half year, however, there had been a number of occasions on which Phil's efficient, friendly, and thoroughly competent exterior masked things going on inside him that were very troublesome. He'd had four or five periods in which he trembled for no apparent reason, feeling tense and jittery and easily startled. Just before and during these times, and also on a few other occasions, he had felt very anxious and apprehensive. He didn't know of

what. There was nothing in particular he was fearful of; it was more a feeling of being worried about everything in particular. At those times he tried to check off, or check upon, the important things in his life—work, family, friends—but the fact that each was OK didn't seem to help. He still felt very apprehensive in kind of a free-floating way. Phil also noticed that, in the last 6 weeks, he'd begun to develop a few persistant physical symptoms—dry mouth, sweatiness, light-headedness, and a pounding heart. He'd always been in perfect health, and these symptoms of he-didn't-know-what made him more anxious than ever. On Monday Phil applied to the Golner Center for psychotherapy. Based on his intake interview and psychological testing, the staff decision was reached to offer to treat his Generalized Anxiety Disorder with psychodynamic psychotherapy.

Psychodynamic psychotherapy has its roots in the writings of Sigmund Freud and the psychoanalytic theory of personality that was the cornerstone of his work. As we shall see, the specific procedures and techniques constituting this therapeutic approach grow quite directly from psychoanalytic beliefs about how personality develops and about how it can become disordered. As Korchin (1976) wrote:

> The foundations for behavior and its problems are set down in childhood through satisfaction or frustration of basic needs and impulses. Because of their potentially central role in regard to these needs, the nature and quality of early relationships with parents, siblings, grandparents, peers, and authority figures . . . are given special emphasis and attention. There is thus a distinct historical flavor to the psychodynamic model and a concomitant focus upon the importance of past rather than present events. (p. 324)

In psychoanalytic thinking, neurotic symptoms were often seen as growing from such conflicted, early childhood experiences and frequently involving the child's sexual needs and feelings. Of special importance in this connection, and of special importance for the key techniques of psychodynamic psychotherapy, Freud proposed that there existed processes (e.g., repression) that kept such conflicted needs and feelings from the individual's awareness. These unconscious needs and feelings resulted in emotions and anxieties the individual might actively seek to avoid recognizing (by use of *defense mechanisms*) or dealing with (by use of *resistance* in the psychotherapy itself). The central goal of psychodynamic psychotherapy thus became the uncovering of these unconscious, childhood-based conflicts ("Making the unconscious conscious") and using the cause-and-effect insights thus gained to both relieve symptoms and reconstruct the individual's personality. How does the psychodynamic psychotherapist proceed? According to Korchin (1976):

> A central task of psychoanalysis is to bring out the hidden, unconscious wishes and conflicts which underlie present symptoms and behavior. An important tool toward this end is *free association*. The patient has to accept the *basic rule*, that he will minimize conscious control and without selection or censor-

> ship he will tell everything that comes to mind. To facilitate free association, distracting stimuli are minimized and relaxation is encouraged. . . . Under these conditions, the patient's communications are more likely to follow the dictates of emotional and unconscious logic rather that that of more conventional rational thought. (p. 327)

In psychodynamic therapy, free association is the main, but not the only route to revealing and understanding unconscious material. Hypnosis was tried early by Freud (Breuer & Freud, 1895) but found to be too unreliable. The analysis of dreams is a second such path, one that Freud described as a "royal road to the unconscious." Both dreams and unintentional slips of the tongue were held to often reflect unconscious material whose expressions was relatively undefended against.

In addition to these means of eliciting unconscious antecedents of contemporary problems, equally central to the psychodynamic psychotherapy process is the development and analysis of the transference relationship. As Freud experienced the very strong positive or negative feelings his patients seemed to hold toward him, feelings he felt were highly out of proportion to what had actually occurred in the therapy itself, he reasoned that:

> . . . such feelings were not generated by the present, but rather were brought forward (transferred) from childhood experiences with key persons, notably the parents. The patient was now acting *as if* the analyst were his father or mother, on his part acting out feelings, wishes, and fantasies of his childhood. (p. 329)

These transference reactions (or, in their most intense form, *transference neuroses*); the patient's free associations, dreams, and slips; and, equally important, the patient's resistances to the therapist's explanations of these materials; are all dealt with by the psychodynamic psychotherapist through interpretation. In a real sense, interpretation is the main active procedure in which the therapist engages. Interpretation means providing the patient with new learning by explaining the unconscious sources of current behavior and, by this means, trying to bring into awareness and better understanding the causes, connections, and meanings.

As can be seen, psychodynamic psychotherapy requires certain cognitive, introspective, and verbal skills of its recipients and, thus, is held by some to have only a narrow range of prescriptive applicability. This is especially true for the more intensive form of psychodynamic psychotherapy, psychoanalysis. For many years, psychodynamic psychotherapy was broadly used for a full range of disorders throughout the United States. Although it continues to be used relatively widely, the emergence of a number of other potent psychotherapies in the last 2 decades; the general absence of supportive research, especially for the psychoanalytic form of psychodynamic psychotherapy; and the restricted range of patients for whom it is applicable, as noted, have combined to result in a progressive decline in the use of the approach, a decline we expect will continue.

Behavior Therapies

Mary couldn't understand what was happening to her. All she knew was that it was getting worse and worse. Then Tuesday morning the answer came. She was straightening up her husband's desk and came across a book he seemed to have been looking at, buried under some papers. The book was *A Guide to Common Emotional Problems*, and a corner of one of the pages was turned down. With both curiosity and a bit of apprehension, Mary opened to the earmarked page and saw a paragraph her husband had bracketed in red:

> *Agoraphobia*. The essential feature is a marked fear of being alone or being in public places from which escape might be difficult or help not available in case of sudden incapacitation. Normal activities are increasingly constricted as the fears or avoidance behavior dominate the individual's life. The most common situations avoided involve being in crowds, such as on a busy street or in crowded stores, or being in tunnels, or bridges, on elevators, or on public transportation. Often these individuals insist that a family member accompany them whenever they leave home.

"Agoraphobia, agoraphobia," Mary repeated to herself "so that's what it's called."

Somehow knowing that the way she'd been acting and feeling the past 8 months had a name made her feel worse. She had become a virtual hermit, and the few times she went anywhere it was always with Carl, sometimes clinging to him for dear life.

On a Monday, after three cancellations, and holding on tightly to Carl's arm, a very anxious Mary Price arrived at Golner for her appointment.

The behavior therapies are a variety of procedures that originated in the laboratory research of the experimental or social psychologist, were shown to effectively change behavior in such research contexts, and have since been applied to the real-world concerns of unhappy, ineffective, or emotionally disturbed individuals. Unlike the psychodynamic therapies, they focus on the present, not the patient's long-term history. Because their fundamental assumption is that *all* behaviors, desirable and undesirable, effective and ineffective, are learned, they emphasize unlearning (the undesirable) and new learning (of the desirable) and not insight regarding unconscious or other historical causes of behavior. The person's current interpersonal and physical environment—what behaviors it stimulates, what behaviors it controls, what behaviors it rewards—is seen as the major cause of the behaviors of concern and, hence, is the major target of change efforts.

Several different behavior therapies have emerged in the past 2 decades, each present oriented, learning based, goal specific, and empirically evaluated. In 1958, Joseph Wolpe published his book *Psychotherapy by Reciprocal Inhibition*, a description of *systematic desensitization* therapy. Designed for phobic patients, it grows from the position that such persons have learned to associate high levels of anxiety with certain stimuli, for example flying in an airplane, handling a snake, seeing a dog approach, being in the dark. This is the original learning to be substituted for by new learning. Such new learn-

ing is accomplished in systematic desensitization by teaching the patient how to deeply relax and, in that relaxed state, because one cannot be both relaxed and anxious at the same time, to imagine ever-closer approach to and involvement with the feared stimulus. Systematic desensitization researchers have frequently found that when the patient can, in a relaxed manner, imagine such contact with an airplane, a snake, a dog, the dark, or whatever, she or he can actually, not in imagination, fly in an airplane, handle a snake, pet a dog, be in the dark, or whatever.

Skinner's experimental laboratory research on operant conditioning began to be utilized in a behavior therapy context in 1959 (Ayllon & Michael, 1959) and has had particularly widespread application with a variety of child and adult institutionalized and community patients. Operant conditioning, also known in its therapeutic application as *contingency management*, involves the provision or withdrawal, respectively, of rewards or reinforcements following those behaviors whose occurrence or frequency one seeks to increase or decrease. By systematic provision of material rewards (food, money, stars, check marks, tokens) or social rewards (praise, attention, approval) following behaviors the therapist or the client wants to increase, the probability that the behavior will recur does in fact increase. The withholding of reinforcement successfully decreases the likelihood any given behavior will occur, and thus an important part of contingency management is to withhold reinforcement following the occurrence of undesirable or inappropriate behaviors. This is typically done by means of ignoring the behavior (extinction), temporarily removing the person from possible sources of reward (time out), or taking away or withholding rewards expected by or already in the person's possession (response cost). With contingency management, the combination of rewarding desirable behaviors and removing rewards following undesirable behaviors (sometimes in combination with such other contingency-management procedures as shaping, fading, and prompting) has proven to be an especially powerful and successful behavior-therapy approach (Agras, 1967; A. P. Goldstein, 1983; Kazdin, 1977).

A particularly important quality of the behavior therapies is the willingness of many persons committed to this approach to submit their interventions to rigorous experimental evaluation and to change the intervention, if appropriate, on the basis of the evaluation's results. In spite of its early wide popularity, in recent years the systematic desensitization approach to alleviating phobias has been increasingly replaced by a different intervention, one which research findings show could be even more potent for such patients. Reflecting quite fully the core commonality of new learning described earlier, in this instance via both modeling and exposure, the behavior therapy called *participant modeling* involves procedures in which:

> Initially, the therapist serves as a model in demonstrating safe interaction with the feared objects or situations, and then guides the client in performing the same sequence of graded activities. As treatment continues, the amount of demonstration, protection and guidance is gradually decreased until the client can confront the feared situations comfortably and effectively. (A. R. Sherman, 1979, p. 43)

According to Bandura (1977b), the developer of this approach, its reliable success grows from several sources. The fear or phobia could be reduced in part by a process called vicarious extinction, when the client sees the therapist directly involved with the feared object with no negative consequences. Also during this demonstration phase of participant modeling, the client could learn both fear-reducing information (about flying, snakes, etc) and skills (how to handle, how to approach). Following demonstration, the therapist could approach the feared object *with* the client (side by side, client's arm on therapist's arm as they reach for the snake, etc.), thus capitalizing on the core commonalities of expectancies and relationship. When the client does actually fly in the airplane or hold the snake and nothing terrible actually happens, a process of direct extinction of fear might occur. With these several factors for change operating, it is not surprising that several investigations have reported clearly positive results for participant modeling as a means of altering phobic behavior (Bandura, Jeffery, & Wright, 1974; Ritter, 1969; Thelen & Fry, 1981).

In addition to systematic desensitization, contingency management, and participant modeling, a number of other behavior therapies have been developed since 1960 and are being widely applied. These include assertiveness training, self-control training, flooding, contingency contracting, stress inoculation, and the several intervention approaches described in our examination of health psychology (chapter 4). Though over its history behavior therapy has at times been misunderstood and suffered from a bad image among both the professional community and the general public, much of that has changed, and the behavior therapies have increasingly come to be seen for what they truly are: effective, reliable, cost-efficient sets of procedures for altering in positive ways a broad array of unhappy, ineffective, or undesirable behaviors.

Client-Centered Psychotherapy

Benny Perez was now well into his third semester after transferring to Drumlin State University, and things seemed to be getting worse and worse. The year he had spent at Colston College hadn't been so hot either, but he thought it had been mostly a problem there of not fitting in with a tight little clique. Now here he was, in a place with over 30,000 other students, and he felt even more isolated and alone. "If I can't make a friend or two here, or get a date, or have

some fun, or feel I fit in somehow . . . " his thoughts ran, and sort of trailed off with no solutions in sight.

Partly, he had transferred to Drumlin in the first place because he'd heard it was a bit of a party school. It had plenty of fraternities, clubs, organizations, and partying, but Benny's attempts during these three semesters to join, or to join in, were one disaster after another. Either he didn't know what to say, and said nothing at all, or he tried to say something—anything—and it came out sounding stupid. The last date he'd had was almost 3 months ago, and, after about an hour and a half, the girl had said she felt sick and asked to be taken back to the dorm. Some success story!

At least for the first two semesters here his grades had held at more or less Benny's B-minus average, but, at midterm this semester, exam grades were running mostly Cs and Ds. Benny really felt like he was going down the tube.

On March 23, Benny called Golner and made an appointment for later that very week.

In clear contrast with the central place given unconscious material and its elicitation in psychodynamic psychotherapy, and with faulty learning and its relearning in the behavioral therapies, client-centered therapy rests on the fundamental belief that individuals experiencing psychological difficulty possess within themselves the resources necessary for dealing with these difficulties and growing beyond them into a more fully functioning, actualized self. It is the therapist's prime responsibility in this approach to provide the conditions, the therapeutic environment, most facilitative of such growth.

Developed by psychologist Carl Rogers, this approach was originally called *nondirective counseling* (Rogers, 1942) to emphasize the client's, rather than the therapist's role in selecting topics for discussion, setting the sessions' tone, and otherwise directing the flow of the therapy. This focus recieved even greater emphasis when Rogers (1951) changed the name of the approach to *client-centered therapy* in 1951. Korchin (1976) captured its tone and substance well:

> The proper therapeutic relation is seen as egalitarian, requiring permissiveness and nondirectiveness, unlike the more formal and authoritarian relation between doctor and patient. Diagnosis is specifically avoided. for it casts the therapist in the role of expert who could discover and tell the patient, conceived as a sick and inadequate person, what was wrong and what to do. . . . Similarly, interpretation just as reassurance, advice, or manipulation, is seen as a directive technique and to be avoided. Such acts all imply that the therapist knows, better than the client, why the client acts as he does and what is good for him. (p. 357)

It is clear from Korchin's comment what the client-centered therapist avoids doing, but what does he or she do? How does client-centered therapy proceed? As early as 1940, Rogers began to make clear that his answer to

this question lay in a growth-facilitating, therapist–client relationship. He commented at that time:

> In the rapport situation, where he is accepted rather than criticized, the individual is free to see himself without defensiveness, and gradually to recognize and admit his real self with its childish patterns, its aggressive feelings, and its ambivalences, as well as its mature impulses, and rationalized exterior. (C. R. Rogers, 1942, p. 162)

Much of the theorizing and wealth of research on client-centered therapy in the subsequent decades of its growth and development has focused on identifying and understanding therapist attitudes that, in actuality, are facilitative of such client growth. Although several such conditions were proposed, three in particular came to be viewed as immensely potent in facilitating client change, so much so that at one point C. R. Rogers described them as the "necessary and sufficient conditions of therapeutic change" (1957, p. 96). This triad of purportedly highly potent therapist attitudes is composed of empathy, unconditional positive regard, and genuineness. Empathy is the experiencing of an accurate understanding of the client's private world, especially his or her emotional world, as if it were the therapist's own. It is taking the client's perspective, fully understanding it, experiencing it in an "as if" manner, and communicating this accurate understanding back to the client. Unconditional positive regard, also called *nonpossessive warmth*, is caring, prizing, or valuing the client in a manner not contingent on the diagnosis, difficulties, or statements during the therapy itself. Genuineness, or congruence, is a purported facilitative condition concerned with the therapist's realness. For the triad of conditions to be facilitative to client growth, the therapist's attitudes of empathy and nonpossessive warmth and the overt behaviors reflective of these attitudes in the therapy sessions themselves had to be genuine, not put on or a facade.

It is to the great credit of Carl Rogers and his students that the now healthy and widespread applied psychological activity of rigorously and scientifically studying the psychotherapeutic process and its outcome was originated by them in the 1940s and early 1950s (Butler, 1958; Covner, 1942; Haigh, 1949; Halkides, 1958; Snyder, 1945). As part of this broad and continuing research program, it has now become clear that, although Rogers (1957) was overly optimistic in his early view that psychotherapy for all types of clients would be greatly facilitated by therapist empathy, unconditional positive regard, and genuineness, it is nevertheless true that, for many psychotherapy patients of several types, such therapist attitudes indeed function to facilitate client openness, disclosure, growth, and, ultimately, movement toward the very type of self-actualization C. R. Rogers envisioned (Barrett-Lennard, 1962; A. P. Goldstein & Michaels, 1985; Parloff, Waskow, & Wolfe, 1978; Rogers, Gendlin, Kiesler, & Traux, 1967).